SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Atezolizumab 840 mg · every 2 weeks · lung cancer

Lung settings: previously treated NSCLC; first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; ES-SCLC maintenance after induction. · NCCP 00592 · version 3 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
In advanced cancer the aim is cancer control and symptom relief; after surgery the aim is to reduce recurrence risk. Confirm your exact treatment setting.

Schedule
Atezolizumab 840 mg is given by intravenous infusion on Day 1 every 2 weeks (14 days). The first infusion lasts about 60 minutes; later infusions may last 30 minutes if tolerated. The visit includes checks and preparation. Adjuvant: maximum 12 months. Advanced NSCLC and ES-SCLC maintenance: until progression or unacceptable toxicity. ES-SCLC requires completed induction; this suite contains no induction chemotherapy.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Atezolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose

Discuss the aim for your exact lung setting, likely benefit, alternatives, important immune risks and how treatment may be paused or stopped. Some side effects need long-term medicines. Ask about fertility, pregnancy, breastfeeding and effects on daily life.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00592 v3.

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